Disability Equality and Reasonable Adjustments Policy

Practice	Kirkcaldy Dental & Implant Clinic, 23–25 Tolbooth Street, Kirkcaldy KY1 1RW
PSM section	Disability Equality › Disability and the Equality Act 2010; Improving access for disabled patients; Access survey
Policy lead	Practice Manager [Name]
Version	1.0 (draft for adoption)
Adopted	[date]
Next review	[date – 12 months after adoption]

1. Purpose

This policy explains how Kirkcaldy Dental & Implant Clinic meets its duties under the Equality Act 2010 to disabled patients, people who come with them, and disabled staff. The aim is that, as far as possible, disabled patients receive our services in the same way and to the same standard as patients who are not disabled.

2. Scope

This policy applies to:

- everyone who works at the practice, including the Practice Owner, associates, hygienists and therapists, dental nurses, reception staff, locums, trainees and contractors
- all services we provide: NHS general dental services under NHS Fife and private treatment, including implants
- all contact with patients: in person, by phone, email, text message (SMS), letters, the website (kirkcaldydental.co.uk), online forms and social media
- the practice as an employer (recruitment and employment of disabled staff).

The Equality Act 2010 defines a disabled person as "someone who has a physical or mental impairment which has a substantial and long-term adverse effect on their ability to carry out normal day-to-day activities". Not all disabilities are visible. They include sight impairments, hearing impairments, physical and mobility impairments, mental health conditions, learning difficulties and some medical conditions (for example diabetes or multiple sclerosis).

3. Legislation and guidance

- Equality Act 2010 (disability is one of nine protected characteristics)
- Government Equalities Office – Equality Act 2010: Disability quick start guide for service providers; Duty on employers to make reasonable adjustments for their staff
- Equality and Human Rights Commission (EHRC) guidance for service providers and employers
- Current Scottish building standards (all rooms and areas in new or refurbished buildings used by staff, patients and visitors must comply)

- NHS Health Facilities Scotland – Access Audit Survey Toolkit (SHFN 02, 2007)
- Disability Rights Commission – Making Access to Goods and Services Easier for Disabled Customers
- Scottish Accessible Information Forum (SAIF) guidelines; Disability Information Scotland – Making Information Accessible; A Brief Guide to Easy Read
- GDC Standards for the Dental Team

PSM pages:

- https://www.psm.sdcep.org.uk/topics/disability-equality/
- https://www.psm.sdcep.org.uk/topics/disability-equality/disability-and-the-equality-act-2010/
- https://www.psm.sdcep.org.uk/topics/disability-equality/disability-and-the-equality-act-2010/general-requirements-for-compliance/
- https://www.psm.sdcep.org.uk/topics/disability-equality/disability-and-the-equality-act-2010/what-are-reasonable-adjustments/
- https://www.psm.sdcep.org.uk/topics/disability-equality/improving-access-for-disabled-patients/ (and its sub-pages: main entrance, steps, doors, emergency exits, layout, WCs, impaired hearing, impaired sight, learning disabilities, changing staff practices)
- https://www.psm.sdcep.org.uk/topics/disability-equality/access-survey/

4. Responsibilities

Practice Owner [Nas] – overall legal responsibility as service provider and employer. The employing dentist is liable for discriminatory acts committed by employees in the course of their work. It is a defence to show that reasonably practicable steps were taken to prevent them, such as disability equality training. The Practice Owner approves the access improvement plan and budget.

Practice Manager [Name] – policy lead. Arranges the access survey, keeps the access improvement plan and the reasonable adjustments log, organises training, and checks this policy is followed.

All clinicians – make reasonable adjustments in the surgery, record patients' communication and access needs in Dentally, and refer or arrange alternative provision where needed.

Reception staff – ask about and record access needs at booking, offer alternative formats and methods of contact, and help patients on arrival.

Fire Safety Lead [Name] – makes sure the evacuation procedure covers disabled patients and staff (see section 5.7).

Every team member – treat all patients with dignity and respect, and never discriminate.

5. Policy and procedure

5.1 What counts as discrimination

Staff must understand the six types of disability discrimination:

1. **Direct discrimination** – treating someone with a disability less favourably than others.
2. **Indirect discrimination** – a rule, policy or practice that applies to everyone but particularly disadvantages a disabled person.
3. **Failure to make reasonable adjustments.**

4. **Discrimination arising from disability** – treatment "connected with" a disability, for example refusing a patient with an assistance dog.
5. **Harassment** – unwanted behaviour related to disability that makes a disabled person feel humiliated, offended or degraded.
6. **Victimisation** – treating someone badly because they have made a complaint of discrimination.

5.2 Our commitments as a service provider

1. We do not refuse a service, provide a lower standard of service, or offer worse terms to anyone for a reason related to their disability.
2. We check that our own policies and procedures (for example failed appointment rules, booking methods, payment arrangements) do not indirectly put disabled people at a substantial disadvantage. Each policy review asks this question.
3. We think ahead. The duty to make reasonable adjustments is anticipatory: we plan for disabled patients before they arrive, not only when asked.
4. Where a physical barrier makes it impossible or very difficult to provide a service at the practice, we provide it another way – for example by arranging a domiciliary visit [if the practice provides domiciliary care] or referring the patient to a practice or service with suitable facilities (for NHS Fife, the Public Dental Service – see Policy 06 – Referrals and Communication with Other Professionals Protocol).

5.3 Deciding what is "reasonable"

The law does not define "reasonable". It is up to the practice to decide, case by case. The duty has three parts:

- changing the way things are done (a policy or procedure)
- changing physical features to overcome barriers (for example a ramp or automatic doors)
- providing an auxiliary aid or service (for example information in an accessible format, an induction loop, an interpreter).

When deciding, the Practice Manager and Practice Owner consider the EHRC factors:

- whether the step would be effective in overcoming the difficulty
- how practicable it is for us to take the step
- the financial and other costs
- the extent of our resources and our size.

If we decide a step is not reasonable, we record why in the reasonable adjustments log, and record any unmet need. We must be prepared to justify the decision.

5.4 Access survey (before opening and when premises change)

1. Before opening, and before any alterations to 23–25 Tolbooth Street, the Practice Manager arranges an access survey by someone experienced or qualified, for example:
   - the local Access Panel (via Disability Equality Scotland) – disabled volunteers who can visit for a small donation or fee
   - Capability Scotland "user access consultation" (small fee)
   - an accredited access auditor from the National Register of Access Consultants (NRAC), or advice from the Centre for Accessible Environments (CAE) – more detailed and more expensive.
2. The survey records features that already help disabled patients and areas for improvement. Use the SDCEP access survey checklist and the Health Facilities Scotland toolkit.

3. We ask disabled patients for their views on access (for example through the patient questionnaire and comments).
4. The Practice Manager turns the findings into an **access improvement plan**:
   - simple measures done immediately (rearranging furniture, clearer signs, keeping the doorway and steps clear)
   - measures added to the building maintenance programme or a future refurbishment
   - a staged timetable so changes are made in the most cost-effective way.
5. Before any physical change, check whether planning permission, building warrant or other consents are needed (contact Fife Council planning/building standards). Structural changes need professional advice.
6. The survey is repeated [every 3 years / when the premises change] and the plan is reviewed yearly.

5.5 Physical access checklist

The Practice Manager checks the following at the access survey and then [monthly] during the premises walk-round:

Main entrance

- The entrance is easy to find from the street, with clear signs. Consider a door painted in a contrasting colour.
- The entrance and approach are well lit, even, firm, not slippery (no moss), and free of obstacles such as bins.
- A call bell or entry phone at a height suitable for wheelchair users. It gives a visible sign (for example a light) that the call has been answered or the door released. [Confirm whether fitted.]
- Accessible parking: [state nearest accessible parking bay; a dedicated bay should be at least 3.6 m wide – ask the access panel].

Steps [delete if level access]

- Steps are clearly marked with contrasting nosings, have non-slip surfaces and are kept clear.
- Handrails on one or both sides.
- Options considered: permanent ramp (needs planning permission), internal ramp, or a full-width, non-slip temporary ramp (with a storage place).
- If the main entrance cannot be made accessible, an alternative entrance is safe, signed, unlocked during opening hours, clear and has a way to alert staff.

Doors

- Doors kept clean and free-moving; opening pressure as light as practicable.
- Thresholds as flush as possible (no more than a 13 mm lip). Entrance mats flush; no bristle matting.
- Handles easy to grip (for example D-shaped), in a contrasting colour, at about 1000 mm from the floor to the centre.
- Glazed doors and large glass panels have safety markings.
- Clear opening width of at least 750 mm for wheelchairs.

Layout

- Well lit, uncluttered, with clear, simple signs in sentence case, supported by symbols, on a contrasting background and not behind glass.
- Contrasting colours to show floors, walls, ceilings and door frames.
- Extra lighting at stairs and changes in level, without glare or shadows.

- Reception desk away from windows so staff are not in shadow (helps lip-readers), with a lower section for wheelchair users to sign documents and talk face to face. If not possible, staff come round from behind the desk.
- Waiting room seating arranged so a wheelchair user can sit beside a companion; some chairs with arms.
- No poorly fitted mats, worn floor junctions or slippery polished floors. Blown bulbs replaced immediately.
- Ground-floor surgery available for patients who cannot use stairs: [Surgery number].

WCs

- Not used for storage; kept clear. Alarm (if fitted) checked [monthly].
- [Wheelchair-accessible WC: yes/no.] If not possible, grab rails, good lighting, contrasting colours, slip-resistant floor, outward-opening door where space allows, and no shiny tiles.

5.6 Communication and information

Asking about needs

1. At registration (new patient form and online form) and when updating the medical history, reception and clinicians ask: "Do you have any access or communication needs we should know about?"
2. Needs are recorded in Dentally as a patient alert/note [specify field], for example "Uses wheelchair – ground-floor surgery", "Deaf – BSL interpreter needed", "Prefers email – screen reader user", "Learning disability – double appointment, carer attends".
3. The alert is reviewed at every visit.

Patients with impaired hearing

- Keep background noise low (sound insulation, quiet areas, soft surfaces where possible).
- Face the patient, make sure your face is lit, and remove your mask when speaking if it is safe to do so.
- Offer to write things down. Offer appointment reminders by SMS or email.
- Book a British Sign Language/English interpreter where needed [via NHS Fife interpreting help: 01592 729130, fife.equalityandhumanrights@nhs.scot (text 07805 800005); Fife Council Deaf Communication Service 03451 55 15 03; private patients: [provider]]. Book interpreters when making the appointment. Do not rely on family members, especially children, to interpret clinical information.
- Consider Makaton for patients who use it.
- Induction loop at reception [and in Surgery X] – technical advice needed to install. Staff know how to switch it on and test it [weekly]. A loop sign is displayed.
- Consider equipment for deaf and hard-of-hearing people, such as a visible light for the door release and visual fire alarm beacons (placed at a height that avoids burns from hot lights).

Patients with impaired sight

- Signs and written information in clear, simple text, in sentence case.
- Large-print versions (at least 16 point, Arial) of the patient information leaflet, medical history form, consent forms, complaints procedure and privacy notice, available on request.
- The website follows Scottish Accessible Information Forum guidance and current web accessibility good practice [web developer to confirm].
- Consider an audio version of the practice leaflet (for example Playback Recording Service).
- Offer appointment reminders by email, which screen readers can read.

- Offer to guide blind or partially sighted patients (staff follow RNIB guidance on guiding). Assistance dogs are always welcome.

Patients with learning disabilities

- Speak clearly and directly to the patient, and check understanding.
- If a carer attends, ask the patient how much they want the carer involved, including whether the carer comes into the surgery.
- Allow extra time and reassurance. Offer a double appointment for patients with more profound learning disabilities, and a quieter time of day if helpful.
- Provide Easy Read information (pictures supporting text) where possible – see Disability Information Scotland and Mencap guidance.
- Consent: follow Policy 03 – Consent Policy, including assessment of capacity and the Adults with Incapacity (Scotland) Act 2000 where relevant.

Other needs

- Mental health conditions, autism, dementia and anxiety: offer the first or last appointment of the day, a quiet place to wait, and a pre-visit to see the surgery if helpful.
- Mobility: allow extra time; offer help but do not assume it is wanted.

5.7 Emergency evacuation

1. The fire and evacuation procedure (Policy 19 – Fire Safety Policy) includes how disabled patients and staff leave the building safely, including any personal emergency evacuation plan (PEEP) for staff who need one.
2. [State aids: for example evacuation chair for upper floor; visual fire alarm beacons; refuge point.]
3. Alarm systems are checked regularly (see the fire log).
4. All staff know the procedure and it is tested at fire drills, including the role of assisting disabled patients.

5.8 Staff practice

All staff are expected to:

- remember that disabilities take many forms and are not always visible
- face patients and avoid covering their mouth when speaking; make sure lighting does not put them in shade
- use clear and simple language and check understanding; be ready to write things down or use other methods
- come out from behind the reception desk if it is not at a comfortable level for a wheelchair user
- offer help on steps or moving around the practice, without assuming it is wanted (staff are covered by the practice's public liability insurance – Practice Owner to confirm cover is in place)
- know the evacuation procedure for disabled patients
- keep corridors and the waiting room clear of bags and boxes
- treat all patients with dignity and respect.

5.9 The practice as an employer

1. We do not discriminate against disabled people in recruitment (advertising, shortlisting, interview, offers, terms) or at any stage of employment (pay, training, promotion, transfer, benefits, discipline, dismissal), and we do not victimise anyone who complains under the Act.

2. Health-related questions, including about past sickness absence, are **not** asked before a job offer is made, unless they are needed to find out whether a person can carry out a task that is intrinsic to the job, or to make reasonable adjustments to the recruitment process. Occupational health clearance (see Policy 30 – Health Clearance and Immunisation Policy) takes place after a conditional offer.
3. Job adverts and interview invitations ask whether the applicant needs any adjustments.
4. We make reasonable adjustments for disabled staff, for example: flexible working and time off for disability-related treatment; information in alternative formats (large print, braille, audio); specialist equipment; and reorganising the working environment.
5. Adjustments for staff are agreed in writing, kept on the confidential staff file, and reviewed at appraisal.

5.10 Complaints

Any patient or staff member who feels they have been discriminated against can complain using the practice complaints procedure (Policy 01 – Feedback, Comments, Concerns and Complaints Procedure) or raise a concern (Policy 07 – Raising Concerns and Whistleblowing Policy). The complaints procedure is available in accessible formats. No one is treated badly for complaining.

6. Records and monitoring

Record	Where kept	Review
Access survey report and access improvement plan	Policy folder / Google Drive "Compliance" folder [restricted access]	Plan reviewed yearly; survey [every 3 years] or when premises change
Reasonable adjustments log (request, decision, reasons, unmet need)	[Google Sheet / paper log]	Practice Manager reviews quarterly
Patient access and communication needs	Dentally patient alerts	At each visit
Staff training records and certificates	Staff files	At appraisal
Minutes of practice meetings where Equality Act compliance is discussed	Meeting minutes folder	At least yearly agenda item
Induction loop tests; WC alarm checks	Premises check log	[Weekly / monthly]
Staff adjustments agreements	Confidential staff file	At appraisal

The Practice Manager reviews complaints and patient feedback for any access issues and reports them at the [quarterly] practice meeting.

7. Training

- All staff receive disability equality training at induction and [every 3 years / annually as an update at a practice meeting]. It covers:
  - the legal duties of the practice and staff under the Equality Act 2010
  - the experiences and needs of disabled people (sight, hearing, mobility, learning disability, mental health, hidden disabilities)
  - how staff can change their practice to improve access (section 5.8).
- Reception staff are trained to use the induction loop, book interpreters and record access needs in Dentally.

- Optional: RNID deaf awareness, RNIB guiding, and basic BSL or Makaton.
- Training certificates and appraisal forms are kept as evidence (this also supports the employer's defence described in section 4).

8. Related policies and templates

PSM templates to download and adapt (https://www.psm.sdcep.org.uk/topics/disability-equality/):

- Disability Policy and Procedures template (Nov 2021, Word)
- Disability Equality checklist / access survey checklist

Related policies in this set:

- Policy 01 – Feedback, Comments, Concerns and Complaints Procedure
- Policy 03 – Consent Policy
- Policy 06 – Referrals and Communication with Other Professionals Protocol
- Policy 07 – Raising Concerns and Whistleblowing Policy
- Policy 08 – Practice Information and Advertising Policy (accessible patient information and website)
- Policy 17 – Health and Safety Policy
- Policy 19 – Fire Safety Policy (evacuation of disabled people)
- Policy 30 – Health Clearance and Immunisation Policy
- Policy 12 – Child and Adult Protection (Safeguarding) Policy

9. Inspection checklist

- ☐ Disability policy in place, signed, dated and read by all staff
- ☐ Access survey completed, with findings recorded and an access improvement plan
- ☐ Evidence of reasonable adjustments made (for example ramp, induction loop, large-print materials, ground-floor surgery) and record of any unmet need with reasons
- ☐ Induction loop (if fitted) working and signed; staff know how to use it
- ☐ Accessible signage; entrance and routes clear and well lit
- ☐ Alternative arrangements for patients who cannot access the premises (referral or domiciliary care)
- ☐ Evacuation procedure covers disabled patients and staff
- ☐ Staff disability equality training recorded; Equality Act discussed at a practice meeting (minutes)
- ☐ Complaints procedure in place and available in accessible formats

Staff sign-off

Name	Role	Signature	Date